Bed Management and Discharge Procedure

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1. Purpose

This procedure explains how beds are requested, assigned, cleaned and released at CareOps General Hospital so that patients move to the right bed without avoidable delay. Good bed management reduces ED boarding, shortens waiting time and improves occupancy balance.

2. Bed status codes

Every bed has one status in the hospital system. Occupied means a patient is in the bed. Vacant clean means the bed is ready for a new patient. Vacant dirty means the patient has left and cleaning is pending. Blocked means the bed is out of use for maintenance or because of a fault. Reserved means the bed is held for a planned admission or transfer. Isolation hold means the bed is held empty after an infectious patient until terminal cleaning is complete. Available beds are those in the vacant clean status and with staff to care for them. The occupancy rate is calculated from staffed beds.

3. Requesting and assigning a bed

The doctor writes the admission decision. The ward nurse or ED nurse sends a bed request to the Bed Manager with the patient department, gender, isolation need and level of care. The Bed Manager assigns a bed within 30 minutes of the request. Assignment rules are as follows. The patient is placed first in the bed of the correct specialty. If that specialty ward is full, the patient may be placed in another ward as an outlier, with a doctor of the correct specialty reviewing the patient daily. Gender and privacy are respected. Patients needing isolation are placed in the Isolation Unit or in a single room. ICU beds are assigned according to clinical need on a priority basis decided by the ICU consultant. The target time from bed assignment to the patient leaving the ED is 60 minutes.

4. Bed huddles

The Bed Manager runs bed huddles at 08:00, 14:00 and 20:00 with the charge nurses of all wards and the ED. Each huddle reviews current occupancy, expected discharges in the next 24 hours, expected admissions from the ED and operating theatres, blocked beds, and beds waiting for cleaning. Decisions and bed numbers are recorded.

5. Bed cleaning and turnaround

Housekeeping is called as soon as the patient leaves. Routine bed cleaning must be finished within 30 minutes. After a patient with an infection needing contact, droplet or airborne precautions, terminal cleaning is carried out and takes 60 to 90 minutes, and the bed stays in isolation hold until the Infection Control rules are met. Beds that stay in vacant dirty status for more than 45 minutes are reported to the Duty Manager.

6. Planned discharge

Discharge planning starts at admission. The doctor records an expected date of discharge within 24 hours of admission and updates it daily. On the day of discharge the doctor completes the discharge order by 10:00 whenever possible. Pharmacy prepares discharge

medicines, nursing completes patient teaching, billing completes the final bill, and transport is arranged. The target is that 30 percent of discharges are completed before 12:00. Patients who are ready to leave but are waiting for family, billing or transport may wait in the discharge lounge so that the ward bed can be cleaned and reused.

7. Reasons for delayed discharge

Common reasons are late doctor review, pending test reports, pending medicines, delays in billing or insurance approval, lack of transport, and waiting for a family member. Each ward records the main reason for every discharge that occurs after 14:00. The Bed Manager reviews these reasons every week and reports the most frequent causes to the Duty Manager.

8. Long stay review

Any inpatient who stays longer than 10 days is reviewed by the department head and the case manager to confirm that the stay is needed and to remove avoidable obstacles. The hospital average length of stay is 4.2 days, and a department whose average is much higher than the hospital average is reviewed.

9. Weekend and night discharge

Discharge activity is lower on weekends and at night, which reduces bed availability on Monday. Consultants therefore review patients who are likely to be discharged on Saturday and Sunday, and pharmacy and billing keep a discharge desk open on those days.